

Appendix C Schedule of Events

	Screening (a,b)	Treatment Cycle (28 Days) (c,d,e)										Follow-up (g)			
		Cycle 1								Cycle 2 and beyond		End of Treatment (f)	EFS Every month	Response FU Every month	OS Every 3 months
		1	2	3	5	8	15 ±1 day	21 ±1 day	1	3	5 ±10 days				
Procedures	Window										+10 days	±1 week	±1 week	±2 weeks	
Informed consent	X														
Inclusion/exclusion (h)	X														
Demographics	X														
Complete medical history and IPSS-R risk categorization (patients with HR MDS or CMML)	X														
Modified Charlson Comorbidity Index assessment (i)	X														
Complete physical examination	X										X				
Symptom-directed physical examination		X								X		X	X		
Height	X														
Weight	X									X(f)		X			
ECOG performance status	X		X(f)							X		X			
Vital signs (k)	X	X		X	X					X	X	X			
12-lead ECG (l)	X											X			
Chest x-ray (m)	X														
Pregnancy test (n)	X	X								X					
Hematology (o)	X	X		X	X	X	X	X	X	X		X	X		
Coagulation (p)	X														
Complete chemistry panel (o,q)	X	X								X		X	X		
Select chemistry panel (o,r)				X	X	X	X	X	X						

Footnotes are on last table page

CONFIDENTIAL

Appendix C Schedule of Events (continued)

	Screening (a,b)	Treatment Cycle (28 Days) (c,d,e)										End of Treatment (f)	Follow-up (g)		
		Cycle 1											Cycle 2 and beyond		
		1	2	3	5	8	15 ±1 day	21 ±1 day	1	3	5		±10 days 22	±1 week	Response FU Every month ±1 week
Procedures	Days	1	2	3	5	8	15 ±1 day	21 ±1 day	1	3	5	±10 days 22	±1 week	Response FU Every month ±1 week	OS Every 3 months ±2 weeks
Window															
Blood phosphate (s)					X						X				
Reticulocyte count and ferritin (t)	X	X							X				X		
Urinalysis with microscopic analysis (u)	X												X		
EORTC QLQ-C30 and EORTC supplemental items (v)	X	X							X				X	X	
EQ-5D-5L (v)	X	X							X				X	X	
Hospitalization assessment (v)		X							X				X	X	
Plasma samples for pevonedistat PK		X (w)			X	X			X	X	(x)				
Whole blood sample for molecular analysis (y)	X											X			
Buccal epithelial cells sample for DNA	X (z)														
SAE collection (aa)	SAEs including serious pretreatment events will be reported from the time informed consent is signed through 30 days after the last dose of any study drug														
AE collection	X (bb)	Recorded from the first dose of any study drug through 30 days after the last dose of any study drug. TEAEs should be monitored until they are resolved or are clearly determined to be due to a patient's stable or chronic condition or intercurrent illness(es).													
Concomitant medications/therapy		Recorded from the first dose of any study drug through 30 days after the last dose of any study drug													
RBC and platelet transfusion documentation	Recorded from 8 weeks before randomization through 30 days after the last dose of any study drug														
Subsequent therapies (cc)													X	X	X
Survival follow-up contact														X	X

Footnotes are on last table page.

CONFIDENTIAL

Appendix C Schedule of Events (continued)

		Treatment Cycle (28 Days) (c,d,e)											Follow-up (g)						
		Screening (a,b)	Cycle 1								Cycle 2 and beyond			End of Treatment (f)	EFS	Response FU	OS		
			Days	1	2	3	5	8	15	21	1	3	5					22	±10 days
Procedures	Window									±1 day	±1 day								
Disease Assessment																			
Disease assessment based on hematology tests (o,dd)													X				X		X
Disease assessment based on bone marrow aspiration/biopsy		See the Bone Marrow Collection and Assessment Schedule Table A																	
Study Drug Administration: Single-Agent Azacitidine or Combination Pevonedistat Plus Azacitidine (e,ee,ff)																			
Pevonedistat infusion			Days 1, 3, and 5 of each cycle																
Azacitidine administration			Days 1-5 and 8 and 9 of each cycle																

AE=adverse event, ALP=alkaline phosphatase, ALT=alanine aminotransferase, AME=acute myelogenous leukemia, aPTT=activated partial thromboplastin time, AST=aspartate aminotransferase, BSA=body surface area, BUN=blood urea nitrogen, CMMI=chronic myelomonocytic leukemia, DME=drug metabolizing enzymes, ECG=electrocardiogram, ECOG=Eastern Cooperative Oncology Group, eCRF=electronic case report form, EFS=event-free survival, EOT=end of treatment, HMA=hypomethylating agents, HRQOL=health-related quality of life, IEC=independent ethics committee, IPSS-R=Revised International Prognostic Scoring System, IRB=institutional review board, IV=intravenous(ly), LDH=lactate dehydrogenase, MDS=myelodysplastic syndromes, OS=overall survival, PK=pharmacokinetic, PT=prothrombin time, RBC=red blood cell, SAE=serious adverse event, SC=subcutaneous(ly), TEAE=treatment emergent adverse events, WBC=white blood cell.

Nonessential protocol visits that do not require on-site sample collection and assessment may be completed via telemedicine (video or phone conversation between the patient and the treating physician, if allowed per Health Authorities, privacy laws, and institutional and local guidelines) in situations where a site visit cannot be conducted, such as in a COVID-19 pandemic.

- (a) Screening assessments will be performed within 28 days before randomization. Baseline assessments are defined as those performed at the closest time before the start of study drug administration.
- (b) Except for hematology, procedures conducted during the Screening period that are performed within 24 hours of Cycle 1 Day 1 can also be used as the baseline evaluation and do not need to be repeated. If dosing falls on a Monday, the collection window may be extended to collect samples on the previous Friday.
- (c) On dosing days, all procedures are to be performed before pevonedistat or azacitidine dosing unless specified otherwise.
- (d) For a new cycle of treatment with study drug(s) to begin, toxicities considered to be related to treatment with study drug(s) must have resolved to the level or grade defined in [Section 8.3.1](#).

CONFIDENTIAL

- (e) The first dose of study drug must be administered within 5 days of randomization on study. It is strongly recommended that dosing for both treatment arms occur on the days specified. However, dosing of either drug may be delayed for safety reasons or other unavoidable circumstances (eg, weather conditions affecting clinic accessibility). If pevonedistat dosing is delayed, a minimum of 1 full calendar day between any 2 doses should be maintained. In each cycle, a maximum of 3 doses of pevonedistat and 7 doses azacitidine (as applicable) should not be exceeded. For the combination arm, pevonedistat and azacitidine should always be administered on the same day (eg, instead of pevonedistat dosing on Days 1, 3, and 5, it would be acceptable to dose on Days 1, 5, and 8). If dosing is adjusted, study procedures should be performed on the actual day of dosing.
- (f) The EOT visit will occur 30 days (+10 days) after the last dose of study drug(s) or before the start of subsequent antineoplastic therapy if that occurs sooner. After the EOT visit, patients with HR MDS or CMML will enter EFS follow-up if their disease has not transformed to AML. Patients with low-blast AML will enter response follow-up if their disease has not progressed (as defined in Section 9.4.20) and they have not started subsequent therapy.
- (g) Patients with HR MDS or CMML will have EFS follow-up study visits every month if their disease has not transformed to AML and they have not started subsequent therapy. Patients who have started subsequent therapy will not be required to have monthly visits but will have a bone marrow aspirate and hematology tests (samples sent to central laboratory) at the time of suspected transformation to AML or PD (for patients who discontinue treatment without evidenced of PD). Patients with low-blast AML will have response follow-up study visits every month until they relapse from CR or meet the criteria for PD. All patients will enter OS follow-up (contacted every 3 months) when they have confirmed transformation to AML (for patients with HR MDS or CMML at enrollment) or experienced PD or relapse from CR (for patients with low-blast AML at study enrollment).
- (h) Confirmation of patient eligibility by the sponsor's project clinician (or designee) is required before randomization. A Patient Eligibility Checklist must be completed and submitted by the investigator for review and approval by the sponsor or designee before patient randomization.
- (i) See [Appendix L](#) for the Modified Charlson Comorbidity Index.
- (j) Weight will be measured within 3 days before Day 1 dosing in each cycle, for calculating BSA. BSA will be calculated using a standard formula (see example in [Appendix M](#)) on Cycle 1 Day 1, and on Day 1 of subsequent cycles if the patient experiences a >5% change in body weight from the weight used for the most recent BSA calculation.
- (k) Vital signs, including diastolic and systolic blood pressure, heart rate, and body temperature will be collected at Screening, predose on Days 1, 3, and 5 on each treatment arm of each treatment cycle, at EOT, and as clinically indicated. Diastolic and systolic blood pressure, and heart rate will be collected postdose on Days 1, 3, and 5 on each treatment arm of each treatment cycle. Vital sign measurements will be taken with the patient in the supine or sitting position.
- (l) Additional ECGs may be performed as clinically indicated.
- (m) If a chest x-ray or chest computed tomography scan was performed within 2 months before randomization, the chest x-ray does not need to be performed during Screening.
- (n) A serum pregnancy test will be performed for women of childbearing potential at Screening. A pregnancy test must also be performed for women of childbearing potential at every cycle (typically performed on Day 1 of the cycle; however, if a serum pregnancy test is used, this may be performed up to 3 days before Day 1) with negative results available before the first dose is administered in that cycle. A pregnancy test will also be performed for women of childbearing potential at the EOT visit. Pregnancy tests may also be repeated during the study if requested by an IEC/IRB or if required by local regulations.
- (o) Clinical laboratory evaluations will be performed by a central laboratory. The central laboratory results also should be used for determination of eligibility by the sponsor's project clinician (or designee) before randomization. For dosing decisions and/or safety concerns, local hematology and chemistry results should be used; however, samples must still be sent to the central laboratory as well. Hematology and chemistry samples may be collected up to 3 days before Day 1 dosing and 24 hours before Days 3 and 5 dosing, when required. Local laboratory evaluations may be done more frequently at the investigator's discretion.
- (p) Coagulation panel includes PT and aPTT (to be performed by the central laboratory).
- (q) The complete chemistry panel will include the following: BUN, creatinine, sodium, potassium, chloride, carbon dioxide, glucose, urate, total bilirubin, direct bilirubin, ALP, LDH, AST, ALT, albumin, magnesium, phosphate, and calcium.
- (r) The select chemistry panel will include the following: BUN, creatinine, total bilirubin, ALP, AST, and ALT.
- (s) Blood phosphate test to be performed on Day 5 of each treatment cycle or the day on which the third dose of pevonedistat is given if it is not Day 5 (to be performed by the central laboratory).

CONFIDENTIAL

- (i) Reticulocyte counts and ferritin level testing will be performed by the central laboratory only. Reticulocyte count and ferritin samples may be collected up to 3 days before Day 1 dosing.
- (u) Urinalysis will include assessments of turbidity and color, pH, specific gravity, protein, ketones, bilirubin, occult blood, nitrite, glucose, and leukocyte esterase. Urine microscopic analysis will include erythrocytes, leukocytes, bacteria, casts, and crystals. These samples will be analyzed by a central laboratory.
- (v) Patient-reported outcomes (HRQOL) and hospitalization assessment (ie, details regarding any hospitalizations since the last assessment) should be completed before any other study procedures are performed or study drug regimen is administered. The EORTC QLQ-C30, EORTC supplemental items, and E-Q-5D-5L will be completed for all patients.
- (w) Combination Pevonedistat Plus Azacitidine Arm only: blood samples (approximately 3 mL each) for the determination of pevonedistat plasma concentrations will be collected during Cycle 1 at the following time points: predose (within 1 hour before azacitidine dosing), Day 1 at the end of the pevonedistat infusion (immediately before stopping the infusion), at 1.5 hours (± 30 minutes) and at 4 hours (± 45 minutes) after completion of the pevonedistat infusion; Day 3 predose (within 10 minutes before azacitidine dosing) and Day 5 predose (within 10 minutes before azacitidine dosing), at the end of the pevonedistat infusion (immediately before stopping the infusion), and before the patient's discharge from the clinic visit (or for patients who are hospitalized, after stopping the infusion following postdose vital sign assessments). The exact date and time of each sample collection and the actual start and stop times of the infusion should be recorded accurately, and particular care should be given to the recording of blood sampling times that occur close to the infusion. Details regarding the preparation, handling, and shipping of samples are provided in the Study Manual.
- (x) Combination Pevonedistat Plus Azacitidine Arm only: blood samples (approximately 3 mL each) for the determination of pevonedistat plasma concentrations will be collected at Cycle 2 and Cycle 4 Day 1 (at the end of the pevonedistat infusion [immediately before stopping the infusion] and 3 hours [± 45 minutes] after completion of the pevonedistat infusion); an additional sample will be collected predose (within 10 minutes before azacitidine dosing) on Cycle 4 Day 3. The exact date and time of each sample collection and the actual start and stop times of the infusion should be recorded accurately, and particular care should be given to the recording of blood sampling times that occur close to the infusion. Details regarding the preparation, handling, and shipping of samples are provided in the Study Manual.
- (y) Whole blood samples for molecular analysis will be obtained at Screening or anytime before the first administration of study drug, within ± 10 days when marrow aspirate for molecular analysis is obtained on Cycle 2, Cycle 4, Cycle 6, Cycle 9, and relapse (see Table A). The 10-day window for collection of this sample allows for it to be obtained along with other blood collection procedures thereby reducing the number of venipunctures. The date and cycle of collection of the blood sample for molecular analysis should be noted on the eCRFs. Whole blood samples will be sent to a central laboratory for molecular analysis.
- (z) Buccal epithelial cell samples will be collected for DNA analysis and for genotyping of DME and/or transporter polymorphisms; procedure is specified in the Study Manual.
- (aa) SAEs will be entered on the eCRF (see Section 10.2). SAEs should be monitored until they are resolved or are clearly determined to be due to a patient's stable or chronic condition or intercurrent illness(es).
- (bb) Nonserious pretreatment events related to study screening procedures will be reported from the time of the signing of the ICF up to first dose of study drug and recorded in the eCRFs.
- (cc) Subsequent therapy is defined as an agent(s) with antileukemic/anti-MDS activity (eg, cytarabine, anthracyclines, purine analogues, and HMAs other than azacitidine). Patients who discontinue study treatment to receive single-agent azacitidine off study would not be counted as receiving subsequent therapy.
- (dd) Hematology tests will also be collected at relapse or PD, and as clinically indicated (see Section 9.4.20 for detailed examples).
- (ee) Combination Pevonedistat Plus Azacitidine Arm only: see Section 8.1.2 for pevonedistat dosing instructions. On Days 1, 3, and 5 when both study drugs are administered, azacitidine will be administered first followed by pevonedistat. Subsequent pevonedistat doses may be reduced because of toxicity in accordance with Section 8.3.
- (ff) All patients will receive azacitidine 75 mg/m² IV or SC on Days 1 through 5 (inclusive), Day 8, and Day 9 of each cycle in accordance with Section 8.1.1. The azacitidine dose may be reduced because of toxicity in accordance with Section 8.3.

CONFIDENTIAL